

PRODUCT LITERATURE

MMT MIXTURE

Each 5 ml contains:

Magnesium Carbonate Light 228.5 mg
Magnesium Trisilicate 228.5 mg
Sodium Bicarbonate 228.5 mg

Preservatives:

Methyl Paraben 0.1%w/v
Propyl Paraben 0.01%w/v

Description

White to off white coloured mixture which separates on standing with peppermint odour and salty taste.

Pharmacodynamic properties

Magnesium Trisilicate Mixture is an antacid with slow neutralising action and mild laxative action.

Pharmacokinetic properties

Magnesium chloride and hydrated silica gel are formed during the neutralisation. About 5% of magnesium is absorbed and traces of liberated silica may be absorbed and excreted in the urine. Any sodium bicarbonate not neutralised in the stomach is absorbed and excreted as bicarbonate and sodium ions in the urine in the absence of a plasma deficit.

Indication

For the relief of hyperacidity.

Route of administration

Oral

Recommended Dose

Adult: 5ml – 20ml, 3 to 4 times a day, after meals and at bedtime

Contraindication

Contraindicated in severe renal failure, hypophosphataemia and in patients who must control sodium intake e.g. congestive heart failure, hypertension, cirrhosis of the liver.

Should not be administered to patients with metabolic or respiratory alkalosis, hypocalcaemia or hypochlorhydria.

Hypersensitivity to any of the ingredients.

Warning and Precautions

The product should be used with caution in patients with fluid retention. In view of the sodium bicarbonate content, the product should also be administered extremely cautiously to patients with renal impairment, to patients receiving corticosteroids or patients with respiratory acidosis, eclampsia, or aldosteronism.

If renal function is impaired hypermagnesemia may result giving the symptoms described under overdose.

Interactions with Other Medicament

Antacids may interact with a number of other drugs by altering their absorption and, sometimes, their elimination, thereby reducing their effectiveness. Antacids may also damage enteric coatings designed to prevent dissolution in the stomach. In order to minimise the risk of interactions, this product should not be taken within two to four hours of other medications (allow at least 4 hours before or 2 hours after erlotinib). Examples of other medications which may be affected include, but are not limited to ACE inhibitors, salicylates e.g. aspirin, atazanavir, azithromycin, barbiturates, bile acids, bisphosphonates, cephalosporin antibiotics, fluoroquinolone antibiotics, chloroquine and hydroxychloroquine, deflazacort, digoxin, dipyridamole, eltrombopag, erlotinib, fexofenadine, gabapentin, iron preparations, isoniazid, itraconazole, ketoconazole, lansoprazole, levothyroxine, lithium, methenamine, mycophenolate, nitrofurantoin, penicillamine, phenothiazines, phenytoin, proguanil, rifampicin, rosuvastatin, sulpiride, tetracyclines, tipranavir, ulipristal (avoid use with antacids). There is a risk of metabolic alkalosis when oral magnesium salts are given with polystyrene sulphonate resins.

Pregnancy and Lactation

The potential risk for humans is unknown. As there is no specific data for this product, it is recommended only be used in pregnancy on the advice of a doctor. Caution should be exercised when prescribing to pregnant women as this product contains sodium.

Side Effects

Magnesium salts may cause diarrhea in some patients. Magnesium carbonate and sodium hydrogen carbonate may cause stomach cramps and flatulence as a result of excess carbon dioxide production. Long-term, excessive use has been associated with the development of silica based renal calculi.

ADR: diarrhea, throat swelling, swollen tongue, swelling face

Symptoms and Treatment of Overdose

Overdose, or excessive or prolonged intake of magnesium containing antacids may give rise to hypermagnesaemia, and excessive administration of sodium bicarbonate may lead to hypokalaemia and metabolic alkalosis, especially in patients with renal insufficiency. Symptoms of hypermagnesaemia include nausea, vomiting, flushing of the skin, thirst, drowsiness, hypotension, confusion, muscle weakness, CNS and respiratory depression, hyporeflexia, peripheral vasodilatation, bradycardia, cardiac arrhythmias, coma and cardiac arrest. Symptoms of hypokalaemia and metabolic alkalosis include mood changes, tiredness, shortness of breath, muscle weakness and irregular heartbeat. Muscle hypertonicity, twitching and tetany may develop, especially in hypocalcaemic patients. Excessive doses of sodium salts may lead to sodium overloading and hyperosmolality. Treatment of mild hypermagnesaemia is usually limited to restricting magnesium intake. In severe hypermagnesaemia, ventilatory and circulatory support may be required. Treatment should consist of the intravenous administration of calcium gluconate injection 10% at a dose of 10 – 20ml, to counteract respiratory depression or heart block. If renal function is normal, adequate fluids should be given to assist magnesium removal from the body. Haemodialysis may be necessary in patients with renal impairment or for whom other methods prove ineffective. Metabolic alkalosis and hypernatraemia can be treated by appropriate correction of fluid and electrolyte balance. Replacement of calcium, chloride, and potassium ions may be of particular importance.

Packing

Packed in plastic container of 120 ml or 200 ml

SHAKE WELL BEFORE USE

Storage Conditions

Store below 30oC in a dry place, protected from direct light. Keep away from reach of children. Jauhkan daripada kanak-kanak

Expiry Date

12months from date of manufacture

Name and Address of Product Registration Holder/ Manufacturer:

TERAPUTICS SDN. BHD. (590500-W)
Lot 10 & 11, PERDA Industrial Park,
Lorong IKS Simpang Ampat B,
14100 Simpang Ampat, S.P.S.,
Pulau Pinang, Malaysia

Name and Address of Distributor:

ZONTRON PHARMACEUTICAL SDN.BHD. (445695-T)
Lot 10 & 11, PERDA Industrial Park,
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14100 Simpang Ampat, S.P.S.,
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Malaysian Drug Registration No. :

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